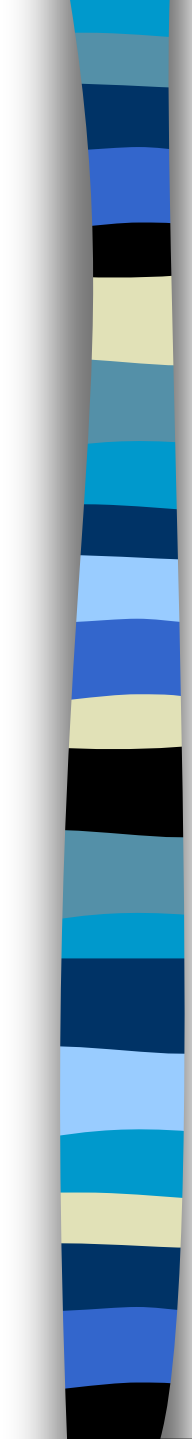


End of Life Care



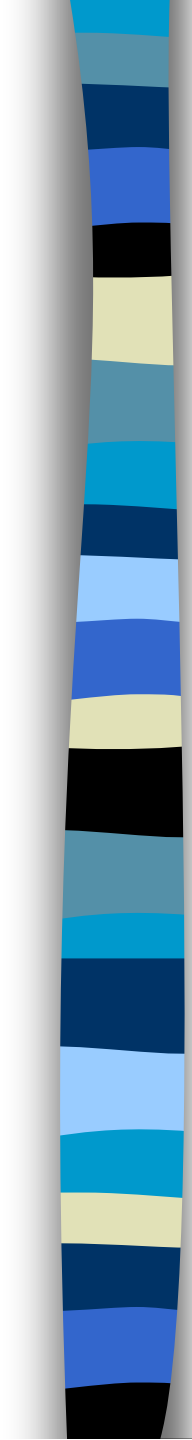
Prepared by:

Dr.: Ons Said Mohamed Elzayat
Assistant professor of community health Nursing



Outlines

- Introduction.
- Definition of death , end of life care & hospice care
- Four goals of end of life care
- Principles of end-of-life care
- End-of-life signs & symptoms.
- The last stage of the end of life care
- Role of the nurse in end of life care



Introduction

End-of-life care refers to the services provided to terminally ill patients whose incurable disease has advanced to a stage near death. Such care may span weeks, months, or even years and applies to multiple disease states. End of life care aims to support a person in the later stages of a life-limiting condition to live as well as possible until they die.

symptom control, spiritual and emotional supports are necessary and require a specialized focus at the end of life.

Definition

Death:

The permanent end of all functions of life in vital organs.

OR The irreversible cessation of all vital functions especially as indicated by permanent stoppage of the heart, respiration and brain activity

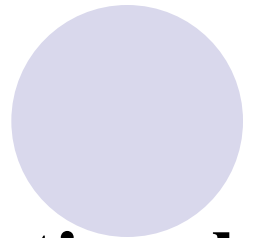
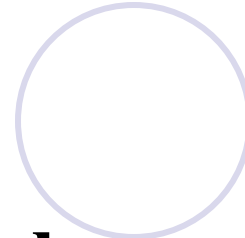
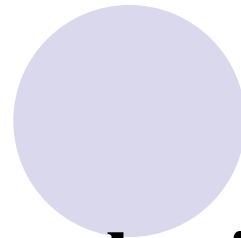
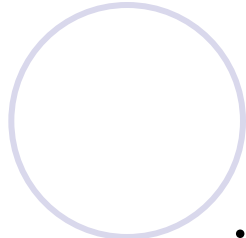
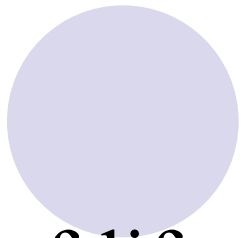
End of Life Care :

It is the care provided to a person in their final stages of life. Also known as hospice care, comfort care, supportive care, palliative care or symptom management. End-of-life care refers to [health care](#). Its broadly care of all person with a [terminal condition](#), that has become advanced, progressive and incurable.

What is end of life care?

End of life care is an important part of palliative care for people who are nearing to the end of life. End of life care is for people who are considered to be in the last period of life. Aims to help people live as well as possible and to die with dignity.





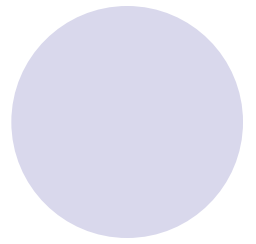
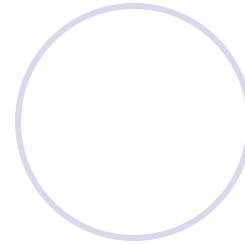
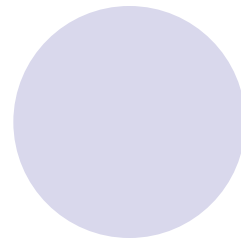
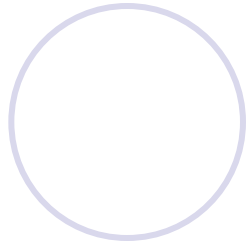
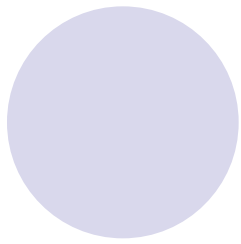
End-of-life care includes **physical, emotional, social, and spiritual support for patients and their families.** The goal of end-of-life care is to control pain and other symptoms so the patient can be as comfortable as possible. End-of-life care may include palliative care, supportive care, and hospice care





Hospice care

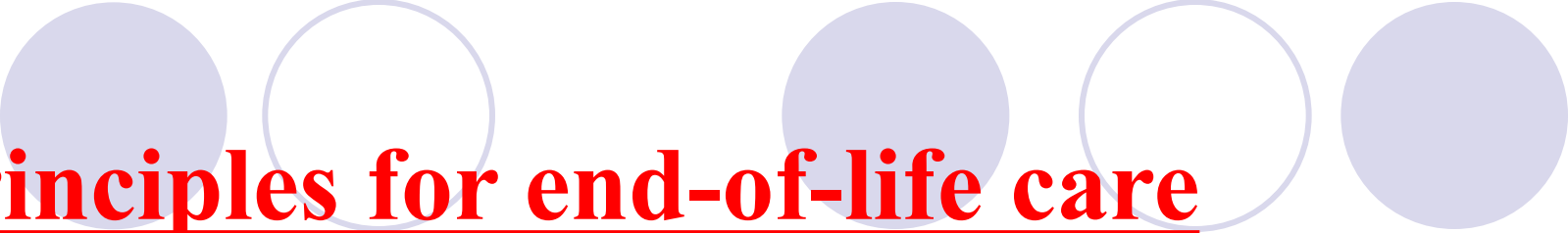
Hospice care is end-of-life care provided by health professionals and volunteers. They give medical, psychological and spiritual support. The goal of the care is to help people who are dying have peace, comfort and dignity. The caregivers try to control pain and other symptoms so a person can remain as alert and comfortable as possible. Hospice programmes also provide services to support a patient's family



Four goals for end of life care

- Physical comfort and relief of suffering
- Mental and emotional health
- Spiritual needs
- Support for families



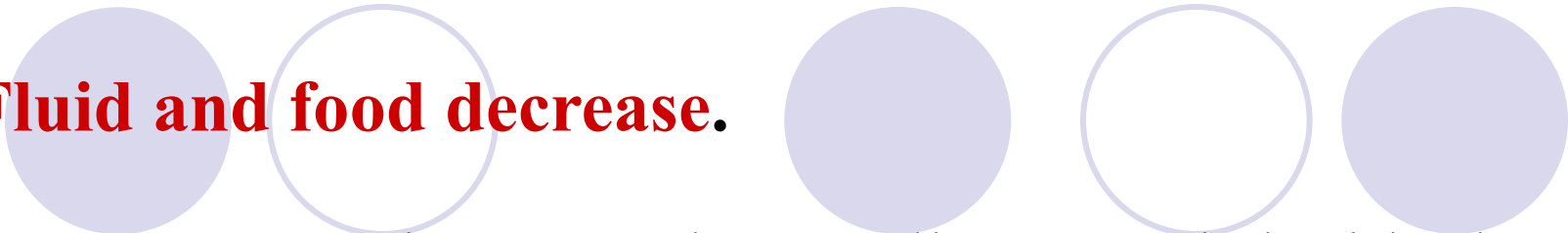


Principles for end-of-life care

- ❑ Respecting the dignity of both patient, caregivers and family's wishes.
- ❑ Alleviation of pain and other physical symptoms

End-of-life signs & symptoms: Physical

- ❑ **Coolness.** Hands, arms, feet and legs may be increasingly cool to the touch. The color of the skin may change.
- ❑ **Confusion.** They may not know time or place and may not be able to identify people around them.
- ❑ **Sleeping.** An increasing amount of time may be spent sleeping.
- ❑ **Incontinence.** They may lose control of urinary/bowel functions. This is a common change that occurs during the end of life process.
- ❑ **Restlessness.** The person may make repetitive motions such as pulling at the bed linen or clothing.



☐ **Fluid and food decrease.**

☐ **Congestion.** There may be gurgling sounds inside the chest.

☐ **Urine decrease.** Output may decrease and become tea colored

☐ **Change in breathing.** The person may take shallow breaths with periods of no breathing for a few seconds to a minute.

Emotional and spiritual signs at the end of life.

❖ **Giving away belongings and making funeral plans:**

Some people want to maintain control over their life, so they want to participate in making final decisions about their belongings

❖ **Withdrawal:** The person may seem unresponsive, withdrawn, in a comatose-like state.

❖ **Vision-like experiences:** The person may say they have spoken or vision to people who are already die.

- ❖ **Restlessness:** Repetitive and restless tasks may indicate something unsolved or unfinished is preventing them from letting go
- ❖ **Communication and permission:** it may mean the person needs your affirmation and support.
- ❖ **Saying goodbye:** There is no need to hide your tears. Tears express your love .

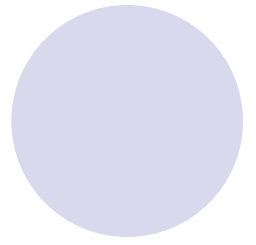
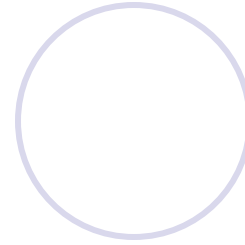
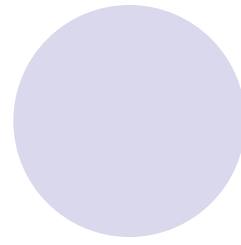
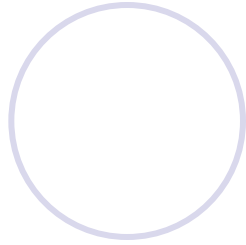
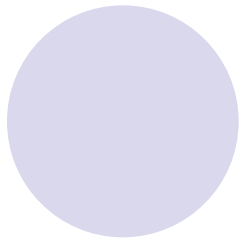


The Last Stages of Life

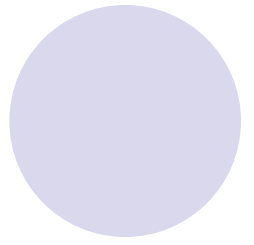
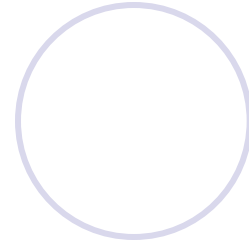
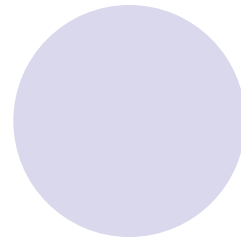
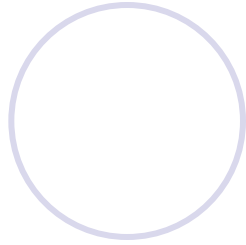
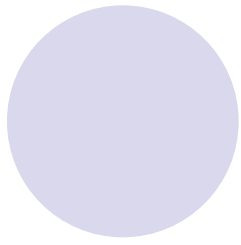
- Withdrawal from the external world.
- Visions and hallucinations.
- Loss of appetite.
- Change in bowel and bladder Functions.
- Confusion, restlessness, and agitation.
- Changes in breathing, congestion in lungs or throat.
- Change in skin temperature and color.

Role of the nurse in end of life care

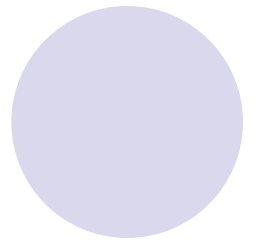
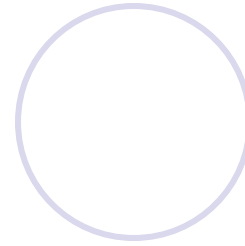
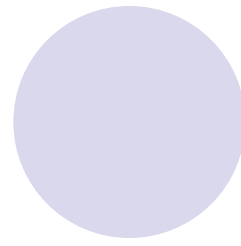
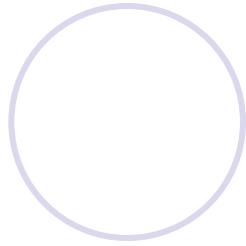
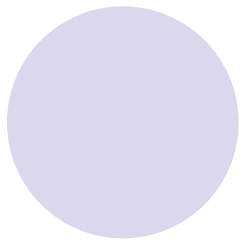
❖ **PSYCHOLOGIST-** Nurses always guide patients through means of effective and therapeutic communication process. Nurses are considered the best emotional supporters



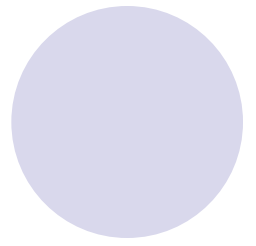
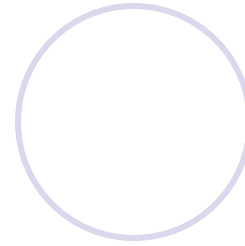
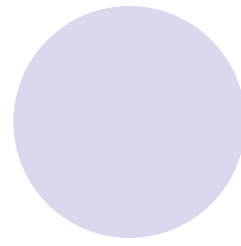
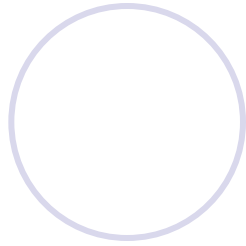
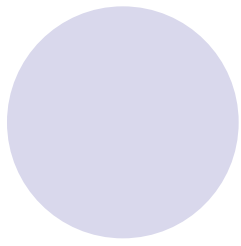
❖ **GUIDANCE COUNSELOR-** A dying patient
conceals the need to be lead always not only from
family but also to others especially to primary care
provider



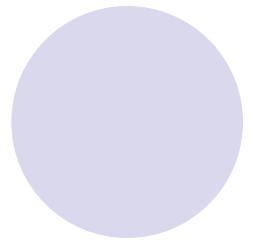
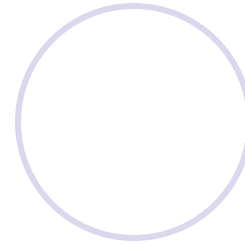
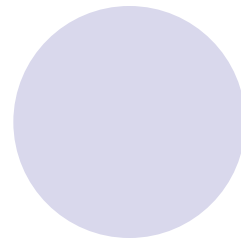
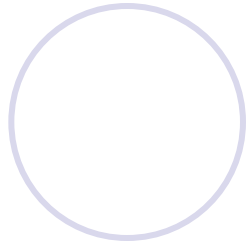
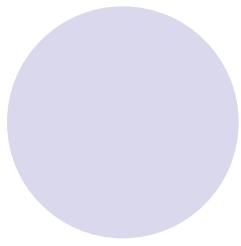
❖ **PREACHER-** All patients regardless of race depends their hope mostly to the need in uplifting their spiritual aspects in life.



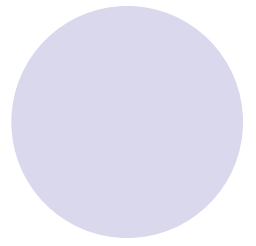
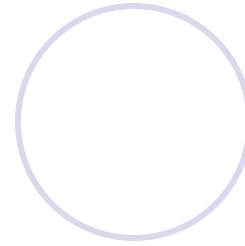
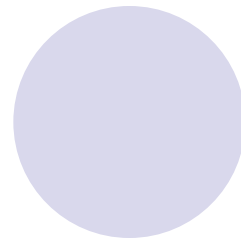
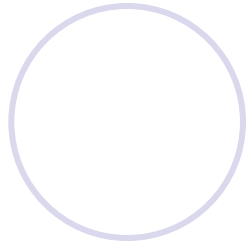
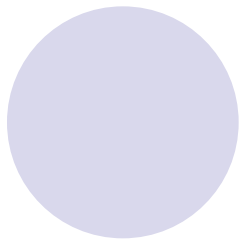
❖ **TEACHER-** Just like children, nurses also serve themselves as teacher especially in giving patients areas of information related to their conditions. Patients are taught in handling and managing their situations by skills that appropriately fit with the care they can conduct with themselves.



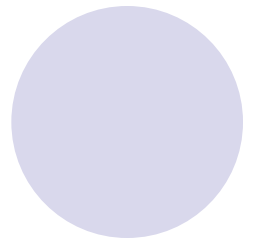
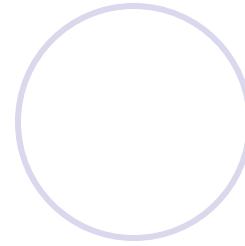
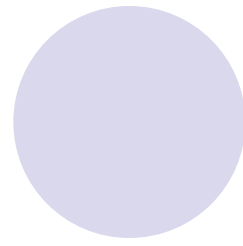
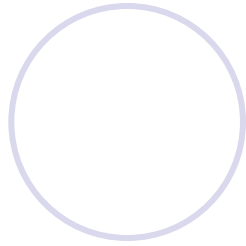
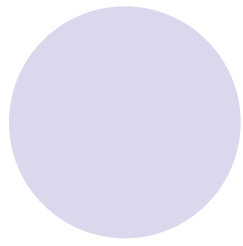
❖ **RELATIVE:** nurses introduce care to all patients just like their siblings, parents or closed relatives. The feeling of the dying patient and the sympathy or grieving of his family is likewise the sympathy and feelings of the nurse.



❖ **COMEDIAN**- This role cannot be applied to all nurses but whenever a nurse has the pride to express a sense of humor, he becomes comedian to the dying patient. This will at least remove or lessen patient's anxiety and depression. Remember that laughter is a reality that sometimes heals patients.



❖ **CAREGIVER-** Nurses are always regarded to be the primary care provider because in a whole shift, their attendance is always on the ward checking and seeing if everyone is in good condition. When a patient suffer from pain, the nurse responds immediately giving any type of care to lessen the discomforts felt by patient.



❖ **TRUSTED MESSENGER**-The nurse is always ready to accept any letters or last words the dying patient wants to express. Most likely to occur with patients unrevealing secrets to their families, the nurse is only the trusted person which that every words from the tongue of the patient makes it an obligation

❖ **RESCUER-** immediate and emergency care from the nurse rather than the doctor because of guarding them in a whole 8-hour work whether at night or day. However, nurse's routine differs. Nurses are rescuers as they become the first line to recover patient's life especially in times of emergency cases.

❖ **MOTHER-** Mothers know best for their children as nurses also knows best for their patients.

Thank You!

Bradford
Jules

